

Policy on Incident Management and Safety Events

«Hospital Name»

Document control

Document No.	HCO/PSQ/POL/07	Version	2.0
Issue No.	01	Review due	One year from implementation
Date created	_____	Date of implementation	_____
Prepared by	Patient Safety Officer Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

A blank marked _____ must be completed before issue.

Statement of intent

«Hospital Name» implements an incident management system covering identification, reporting, review and action on incidents. The system supports factual reporting and learning and is based on the principle of just culture.

The organisation identifies relevant sentinel events, analyses incidents, takes corrective and preventive action, manages unidentified risks revealed through incident analysis, and informs relevant stakeholders after due analysis and classification.

1. Purpose

The purpose of this policy is to establish the process for identification, reporting, review, analysis and management of incidents and relevant safety events.

This policy does not cover matters outside incident management.

2. Scope

This policy applies to incidents occurring within «Hospital Name» and to personnel involved in incident reporting, review, analysis, corrective and preventive action, risk management and stakeholder communication.

It includes sentinel events, near misses and adverse events as described in the source requirements.

3. Policy standards

3.1 Incident management system

The organisation implements an incident management system covering identification, reporting, review and action on incidents.

The system supports factual reporting and learning and is based on the principle of just culture.

The organisation shall have a mechanism for reporting the occurrence of incidents on standardised incident report forms.

While capturing incidents, the organisation shall capture all incidents without going into the severity or whether harm was caused.

It is preferable that the reporting system is simple, clear, confidential and focused on process improvement.

A near-miss is an unplanned event that did not result in injury, illness or damage, but had the potential to. An adverse event is an injury related to medical management — including diagnosis, treatment, or the systems and equipment used to deliver care — rather than to the patient's underlying disease; adverse events may be preventable or non-preventable (NABH Guidebook, Glossary of Terms).

3.2 Identification of sentinel events

Sentinel events relating to system or process deficiencies that are relevant and important to the organisation must be clearly defined.

The list of identified and relevant sentinel events shall be documented.

A sentinel event is an unanticipated occurrence involving death or serious physical or psychological injury, not related to the patient's underlying illness, but related to the medical equipment, supplies or care provided (NABH Guidebook, Glossary of Terms).

3.3 Analysis of incidents

The safety committee shall be responsible for incident analysis.

Root-cause analysis is the preferred method.

Root-cause analysis is a structured, system-based review of a critical incident: it identifies the root and contributory factors behind what happened, sets out risk-reduction strategies, and builds an action plan with a way to measure whether that plan actually worked.

Inputs could be sought from the units, disciplines or departments concerned.

Where possible, patients and other stakeholders could be included in analysing the feedback and complaint.

The immediate response to a safety incident shall be to address the urgent care and support needs of those involved. This shall not await analysis.

For sentinel events, correction, if any, shall be initiated within 24 working hours of occurrence or reporting.

The analysis of sentinel events shall be completed within seven working days of occurrence or reporting.

3.4 Corrective and preventive action

All corrective and preventive action based on incident analysis shall be documented.

The findings and recommendations arrived at after analysis shall be communicated to all personnel concerned to correct systems and processes and prevent recurrences.

Any change in policy or procedure shall be reflected as an amendment in the organisation's documentation.

3.5 Risk management

If incident analysis reveals the presence of an unidentified risk, the same shall be subjected to risk management.

3.6 Stakeholder communication

After due analysis, an incident could be termed as a near miss, adverse event or sentinel event.

Based on the nature of the near miss, adverse event or sentinel event, the organisation shall inform the stakeholders, including patient and family where applicable, regarding the relevant concerns in addition to initiating corrective and preventive action.

4. Non-negotiable rules

- Do not operate the incident management system without identification, reporting, review and action on incidents.
- Do not report incidents without using the organisation's standardised incident report forms.
- Do not exclude an incident from capture based on its severity or whether harm was caused.
- Do not leave relevant and important sentinel events undefined or their identified list undocumented.
- Do not allow the immediate care and support response to a safety incident to wait for incident analysis.
- Do not initiate sentinel-event correction later than within 24 working hours of occurrence or reporting.

- Do not complete sentinel-event analysis later than within seven working days of occurrence or reporting.
- Do not leave corrective and preventive action undocumented.
- Do not communicate incident-analysis findings and recommendations only to the immediate team; they shall be communicated to all personnel concerned.
- Do not leave changes to policy or procedure unrecorded as amendments in the organisation's documentation.
- Do not leave an unidentified risk revealed by incident analysis outside the risk management system.
- Do not inform stakeholders about a near miss, adverse event or sentinel event before due analysis and classification; the required stakeholder communication occurs after due analysis.

5. What we do

5.1 Operate the incident management system

The organisation operates an incident management system covering identification, reporting, review and action.

Standardised incident report forms are used for reporting the occurrence of incidents.

All incidents are captured regardless of severity or whether harm was caused.

The reporting system preferably remains simple, clear, confidential and focused on process improvement.

5.2 Identify sentinel events

The organisation defines sentinel events relating to system or process deficiencies that are relevant and important to the organisation.

The identified and relevant sentinel-event list is documented.

The definition of sentinel events is defined in the NABH Guidebook to Accreditation Standards for Hospitals, 6th Edition, Glossary of Terms.

5.3 Analyse incidents

The safety committee is responsible for incident analysis.

Root-cause analysis is the preferred method.

Inputs could be obtained from the concerned units, disciplines or departments.

Where possible, patients and other stakeholders could be included in analysing feedback and complaints.

The immediate response to a safety incident addresses urgent care and support needs and does not await analysis.

For sentinel events, correction, if any, is initiated within 24 working hours of occurrence or reporting.

Sentinel-event analysis is completed within seven working days of occurrence or reporting.

5.4 Take corrective and preventive action

Corrective and preventive action based on incident findings is documented.

Findings and recommendations are communicated to all personnel concerned.

Systems and processes are corrected to prevent recurrences.

Changes to policies or procedures are reflected as amendments in organisational documentation.

5.5 Manage unidentified risks

When incident analysis reveals an unidentified risk, that risk is subjected to risk management.

5.6 Inform stakeholders

After due analysis, an incident could be termed a near miss, adverse event or sentinel event.

Based on the nature of the event, relevant concerns are communicated to stakeholders, including patient and family where applicable, in addition to initiating corrective and preventive action.

6. Stop-work authority

6.1 Incident management system trigger

Absence of an implemented incident management system is CORE non-compliance. The incident management system must include identification, reporting, review and action on incidents. Failure to have an implemented system covering these components constitutes the specified stop-work trigger.

7. Governance and responsibility

Role	Responsibility
Management	Ensures implementation of the incident management system and the processes described in this policy.
Safety committee	Shall be responsible for incident analysis.
Personnel involved in reporting and review	Capture all incidents and participate in the

	incident management process.
Personnel concerned with corrective and preventive action	Receive findings and recommendations arising from incident analysis and participate in correction of systems and processes to prevent recurrences.
Risk management responsibility	When incident analysis reveals an unidentified risk, the risk is subjected to risk management.
Stakeholder communication responsibility	The organisation informs stakeholders, including patient and family where applicable, after due analysis and classification of the incident.

8. Quality monitoring

Monitoring area	What is monitored
Incident management system	Identification, reporting, review and action on incidents
Incident reporting	Use of standardised incident report forms and capture of all incidents regardless of severity or harm
Sentinel events	Organisation-defined relevant sentinel events and documented list
Immediate safety response	Urgent care and support provided without awaiting analysis
Sentinel-event correction	Correction initiated within 24 working hours of occurrence or reporting
Sentinel-event analysis	Analysis completed within seven working days of occurrence or reporting
Corrective and preventive action	Documentation, communication to all personnel concerned and documented amendments to policies or procedures
Risk management	Unidentified risks revealed through incident analysis subjected to risk management
Stakeholder communication	Stakeholders informed after due analysis and classification, including patient and family where applicable

9. Training and staff acknowledgement

Staff involved in incident reporting, review, analysis, corrective and preventive action, risk management and stakeholder communication shall be familiar with the processes described in this policy.

I have read the Policy on Incident Management and Safety Events of «Hospital Name». I will follow the processes described.

Staff name	Designation	Signature	Date
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____

10. Distribution

This policy shall be available to personnel involved in incident management and related patient safety activities.

The processes described in this policy shall be communicated to relevant personnel.

11. Abbreviations

Abbreviation	Meaning
CAPA	Corrective and Preventive Action

12. Traceability table

This table is an index. It is not how the policy is organised. An asterisk in the Level column means documentation of the process is required.

Objective Element	Level	Traceability to this policy
PSQ.7.a	CORE*	Sections 3.1, 4, 5.1 and 6 address the incident management system, its four components, standardised incident reporting, capture of all incidents and the specified CORE stop-work trigger.
PSQ.7.b	Commitment*	Sections 3.2 and 5.2 address organisation-defined relevant sentinel events and documentation of the identified list.
PSQ.7.c	Commitment	Sections 3.3 and 5.3 address incident analysis, safety committee responsibility, immediate care and support, and the exact sentinel-event correction and analysis timeframes.
PSQ.7.d	Commitment	Sections 3.4 and 5.4 address documented corrective and preventive action, communication to all personnel concerned and

		documented amendments to policies and procedures.
PSQ.7.e	Achievement	Sections 3.5 and 5.5 address unidentified risks revealed through incident analysis and their submission to risk management.
PSQ.7.f	Commitment	Sections 3.6 and 5.6 address stakeholder communication after due analysis and classification, including patient and family where applicable.

13. Required Records/Evidence Checklist

Incident management and reporting

- Implemented incident management process covering identification, reporting, review and action on incidents.
- Standardised incident report forms.
- Incident reports showing that all incidents are captured regardless of severity or whether harm was caused.
- Incident reporting process supporting factual reporting, learning and just culture.
- Reporting process characteristics showing simplicity, clarity, confidentiality and focus on process improvement where these have been adopted.

Sentinel events

- Organisation-defined list of relevant and important sentinel events relating to system or process deficiencies.
- Document showing that the identified sentinel-event list is defined and maintained.
- Reference to the NABH Guidebook to Accreditation Standards for Hospitals, 6th Edition, Glossary of Terms for the definition of sentinel events.

Incident analysis and response

- Safety committee responsibility for incident analysis.
- Incident-analysis records.
- Records of immediate care and support provided after safety incidents without waiting for analysis.
- Sentinel-event correction records showing initiation within 24 working hours of occurrence or reporting.
- Sentinel-event analysis records showing completion within seven working days of occurrence or reporting.
- Root-cause analysis records where this approach has been used.

- Inputs from concerned units, disciplines or departments where these have been obtained.
- Patient and other stakeholder participation in analysis where this has been used.

Corrective and preventive action

- Documented corrective and preventive actions arising from incident analysis.
- Communication of findings and recommendations to all personnel concerned.
- Policy or procedure amendments made following incident analysis.

Risk management

- Incident-analysis records identifying any previously unidentified risk.
- Risk-management records for unidentified risks revealed through incident analysis.

Stakeholder communication

- Incident classification after due analysis as a near miss, adverse event or sentinel event where applicable.
- Communication records showing relevant concerns were shared with stakeholders, including patient and family where applicable.

14. References

1. National Accreditation Board for Hospitals and Healthcare Providers. NABH Accreditation Standards for Hospitals, 6th Edition. PSQ.7.
2. National Accreditation Board for Hospitals and Healthcare Providers. Guidebook to NABH Accreditation Standards for Hospitals, 6th Edition. Glossary of Terms. Definitions of sentinel events and related terms.

Disclaimer

This document is a template prepared for the guidance of «Hospital Name» and must be reviewed, adapted and formally approved by «Hospital Name» before use. Every entry marked [Hospital to define] must be replaced with the hospital's own decision; a document issued with those markers left in place is not an approved policy.

The requirements in this document are accreditation requirements of the NABH Full Accreditation Standards for Hospitals, 6th Edition, not duties under a named Act of Parliament. This policy does not import the Consumer Protection Act, 2019, the Clinical Establishments Act, 2010, or the Mental Healthcare Act, 2017 as a checklist. Statutory duties that arise under other documents of «Hospital Name» remain those documents. «Hospital Name» is responsible for verifying any statutory duty that applies to it; this document does not constitute legal advice.

The clinical and technical content reflects recognised national and international guidance current at the date of preparation. «Hospital Name» remains responsible for verifying that it

is current and consistent with the edition of the accreditation standard against which it is being assessed.

This document is not issued by, endorsed by, or affiliated with NABH or any other body named in it. Wording is original; no text has been reproduced from the standards, rules or guidelines referenced.